

The next step, accounted for.

A follow-up ledger for the days after discharge.

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Why this matters

Leaving hospital can leave unfinished work: a pending result, a repeat test, or a referral. In a 2005 study of 2,644 discharges at two academic hospitals, 41% of patients had results return after discharge.¹

What the MVP does

Review the source. Paste text or import a text-based PDF. A trained classifier and rules suggest actions. Users review every sentence and confirm the plan.

Name the tracker. Each task keeps its supporting text and date wording. Unclear details remain questions for the care team.

Record the ending. Result receipt stays separate from user-reported clinician review. The history retains reported outcomes.

A plan people can use

Signed-in spaces save plans. A printable brief and calendar/JSON exports keep the record portable. The Anita/Maya demonstration uses fictional data.

AI you can inspect

82% raw accuracy and **0.816 macro F1** on 50 separately authored synthetic challenge examples, after training on 225 synthetic examples. The threshold abstained on **44 of 50**.

These are sentence-classification results, not clinical validation. Human review remains essential. No paid model API is required.

A business to test

First buyer hypothesis: a primary-care or transitions coordinator already chasing discharge follow-up.

\$149/month for 100 care episodes is a pricing hypothesis. Measure preparation time, corrections, and willingness to pay.

The next proof

A supervised pilot should measure missed actions and follow-up resolution. The MVP has no EHR integration or proven health benefit. It carries documented instructions and does not interpret results or make treatment decisions.

MVP for synthetic demonstration. Patient deployment requires clinical and security review.

¹ Roy et al., *Annals of Internal Medicine* (2005). Historical two-hospital finding, not a current global estimate.

Workflow context: [2025 SAFER Test Results Reporting and Follow-Up Guide](#). This does not endorse Looplight.